differing dosing regimens demonstrating IGA success versus 55% of placebo patients. However, contrary to the claims and presentations in the TV ad, experiencing clear skin and maintaining it is not a guarantee for any, let alone "many" patients at any timepoint. The proportion of Nemluvio patients with IGA success was well less than 50% at Week 16 and only represented a 12% difference from placebo across the two studies. Furthermore, the maintenance period data represents a subset of IGA responders at Week 48 out of the IGA responder subset from Week 16. Thus, the interpretability of the 48-week IGA data is only applicable to a limited population and does not support broad efficacy claims for "many" patients.

Conclusion and Requested Action

For the reasons described above, the TV Ad misbrands, Nemluvio and makes the distribution of the drug in violation of the FD&C Act.

This letter notifies you of our concerns and provides you with an opportunity to address them. FDA requests that Galderma take immediate action to address any violations (including, for example, ceasing and desisting promotional communications that are misleading as described above). Please submit a written response to this letter within 15 working days from the date of receipt, addressing the concerns described in this letter, listing all promotional communications (with the 2253 submission date) for Nemluvio that contain representations like those described above, and explaining your plan for the timely discontinuation of such communications, or for ceasing distribution of Nemluvio.

If you believe that your products are not in violation of the FD&C Act, please include in your submission to us your reasoning and any supporting information for our consideration within 15 working days from the date of receipt of this letter.

The concerns discussed in this letter do not necessarily constitute an exhaustive list of potential violations. It is your responsibility to ensure compliance with each applicable requirement of the FD&C Act and FDA implementing regulations.

Please direct your response to the **Food and Drug Administration, Center for Drug Evaluation and Research, Office of Prescription Drug Promotion, 5901-B Ammendale Road, Beltsville, Maryland 20705-1266**. A courtesy copy can be sent by facsimile to (301) 847-8444. Please refer to MA 57 in addition to the BLA number in all future correspondence relating to this particular matter. All correspondence should include a subject line that clearly identifies the submission as a Response to Untitled Letter. You are encouraged, but not required, to submit your response in eCTD format. All correspondence submitted in response to this letter should be placed under eCTD Heading 1.15.1.6. Additionally, the response submission should be coded as an Amendment to eCTD Sequence 6054 under BLA 761391. Questions related to the submission of your response letter should be emailed to CDER-OPDP-RPM@fda.hhs.gov.

This is a representation of an electronic record that was signed electronically. Following this are manifestations of any and all electronic signatures for this electronic record.

/s/

CARTER M BEACH
09/09/2025 05:12:01 PM
On behalf of George Tidmarsh, M.D., Ph.D